SRR - Site Risk Register

Purpose and risk categories

v1.0

Study / protocol	CIP version / date	Site / code	SRR ID / revision
Principal Investigator	**Register owner / role**	**Sponsor risk information (ID / version)**	**Study phase**
Date opened	**Last review / next review**	**Current reviewer / role**	**Controlled location**

A Purpose and use

The SRR records the risks that this site's own way of working creates for the investigation: gaps in staff, workflow, equipment or experience that could compromise a condition critical to quality (CtQ). The sponsor's risk assessment and the manufacturer's device risk file cannot see these from outside, and the SRR does not copy them. Open it before the first participant is included and keep it current until close-out.

How the form works

- **Pages 1 and 2** are reference: risk categories, CtQ domains and the rating guide.
- **E and F** are the register: one row per risk, with the same Risk ID on both pages. Reprint the pair when it fills.
- **G** records every review of a risk; **H** captures an urgent risk before it enters the register. Each is reprinted on its own.
- **I** holds the signed decisions to lower, accept, close or escalate a risk; **J** closes each review of the whole register.

Unit of analysis	Decision supported	Inputs	Outputs
One local risk to a CtQ factor of this investigation	What threatens the factor, what risk remains with the controls in place, what happens next, who owns it and when it is reviewed again.	CIP, sponsor risk information, local workflow and capacity, and findings from the IMC, PIR, CSST, SLCT, SART and CAPA log.	Controls to verify (IMP) PI decisions (POT) Phase-gate conditions (MRL) CAPA, sponsor and SART routes

B Risk categories

Risk category	Typical local vulnerability	What may be affected
Safety / clinical event	Delayed safety awareness, incomplete assessment or missed reporting.	Participant protection and timely clinical decisions.
Procedure / surgeon performance	Learning-curve drift, unplanned substitution or inconsistent technique.	Procedural consistency and interpretation of outcomes.
Device accountability	Mixed or poorly segregated stock, missing identifiers or reconciliation gaps.	Component selection and participant-device traceability.
Data / endpoint reliability	Missing assessment, unresolved critical query or source-to-CRF mismatch.	Complete, reliable and interpretable study data.
Operational / staffing	No assessor on needed days, turnover, workload or unclear handover.	Capacity to deliver required study activities.
Consent / eligibility	Wrong consent version, delayed re-consent or unsupported eligibility.	Participant rights and valid enrollment.
Imaging / assessment workflow	Wrong projection, missed window or inconsistent assessment method.	Comparable and interpretable endpoint assessments.
Vendor / external data	Delayed external records, inaccessible results or missing transfers.	Availability of safety and endpoint evidence.

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CtQ domains and rating guide

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C CtQ domains

In the register, give the domain code and write the concrete factor of this investigation that the risk threatens, with its source (CIP, sponsor risk information, local workflow). A domain name is not a factor.

Code	CtQ domain	What it covers
CtQ1	Site capability, readiness and capacity maintenance	Staff, volume, facilities, equipment, imaging and follow-up capacity
CtQ2	Participant selection and baseline characterization	Eligibility criteria, baseline assessment, documented selection decision
CtQ3	Informed consent and participant understanding	Consent process, version control, comprehension, re-consent on amendment
CtQ4	Device management, accountability and traceability	Receipt, storage, use in a named participant, identifiers, reconciliation
CtQ5	Procedural delivery and surgical technique standardization	Approach, positioning, soft tissue, intraoperative decision thresholds
CtQ6	Training, credentialing and learning-curve control	Competency, device release, maintained authorization, learning effects
CtQ7	Perioperative and postoperative care pathway and follow-up retention	Care pathway, visit scheduling and windows, retention
CtQ8	Endpoint framework and outcome-assessment standardization	Assessor consistency, administration conditions, PRO timing and mode
CtQ9	Imaging acquisition, interpretation and endpoint control	Acquisition parameters, operator consistency, local review, timing
CtQ10	Safety surveillance and adverse device event management	Detection, classification, causality and reporting of AEs and DDs
CtQ11	Data capture, source documentation and data-flow integrity	Source adequacy for reconstruction, source-to-record consistency
CtQ12	Regulatory compliance and deviation management	Detection, documentation and correction of deviations

D Risk rating guide

Rate with the controls already in place; a planned action does not lower a score. Write N/A, with a short reason, when a dimension does not apply. Do not average or multiply. A reportable event follows its CIP route at every priority.

Rating field	1	2	3
Probability	Unlikely or isolated vulnerability.	Plausible in normal site conduct.	Likely, recurrent or already observed.
Safety	No expected effect on safety or participant protection.	May affect timely clinical review, protection or safety documentation.	May cause or hide serious harm, a serious device effect or a major safety concern.
Procedure	No expected effect on the conduct of the procedure.	May affect consistency, documentation or protocol adherence.	May affect surgical appropriateness, intraoperative decisions or technique.
Device	No traceability or selection concern.	Recoverable discrepancy or delayed reconciliation.	Missing or conflicting identifiers, wrong component or broken participant-device link.
Data	No expected effect on critical data.	May affect completeness, timeliness or source-to-CRF consistency.	May compromise endpoint interpretation, eligibility evidence or critical data.
Detection	Easy: found by routine checks before it has an effect.	Moderate: found only at a planned review, reconciliation or second check.	Difficult: may stay hidden until endpoint review, inspection or recurrence.

Low	Moderate	High	Critical
All impact dimensions are 1, probability is low and the problem is easy to detect.	At least one dimension is 2, or probability is medium, but no dimension is 3 and controls appear adequate.	Any dimension is 3, or the problem recurs, is poorly controlled, hard to detect or needs active PI review.	Immediate or unresolved safety risk, broken participant-device link, unrecoverable endpoint data, or an event to escalate now.

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Main register · identification and rating

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Study / site	SRR ID / revision	Revision effective date	Register sheet no. / total

E Main register · identification and rating

Name the domain, the concrete factor threatened and its source; describe the local vulnerability and its consequence; list the controls already in place with their evidence; then rate. Continue the same Risk ID in F.

Risk ID	Date opened	Category	CtQ domain, factor and source	Local vulnerability and possible consequence	Existing controls and evidence	Prob.	Safety	Proc.	Device	Data	Detect.	Priority
			Domain: Factor: Source:									☐ Low ☐ Moderate ☐ High ☐ Critical
			Domain: Factor: Source:									☐ Low ☐ Moderate ☐ High ☐ Critical
			Domain: Factor: Source:									☐ Low ☐ Moderate ☐ High ☐ Critical
			Domain: Factor: Source:									☐ Low ☐ Moderate ☐ High ☐ Critical
			Domain: Factor: Source:									☐ Low ☐ Moderate ☐ High ☐ Critical
			Domain: Factor: Source:									☐ Low ☐ Moderate ☐ High ☐ Critical
			Domain: Factor: Source:									☐ Low ☐ Moderate ☐ High ☐ Critical

SRR - Site Risk Register

Main register · ownership, action and status

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F Main register · ownership, action and status

Risk status says whether the risk still applies; action status says where the response stands. Implemented is not Effective until a review in G shows the control works.

Risk ID	Risk owner (role)	Review trigger or threshold	Planned action / owner / due	Linked records and evidence	PI / sponsor route and deadline	Risk status	Action status	Last / next review
						☐ Open ☐ Controlled ☐ Accepted ☐ Closed	☐ Not started ☐ In progress ☐ Implemented ☐ Effective ☐ Not effective	
						☐ Open ☐ Controlled ☐ Accepted ☐ Closed	☐ Not started ☐ In progress ☐ Implemented ☐ Effective ☐ Not effective	
						☐ Open ☐ Controlled ☐ Accepted ☐ Closed	☐ Not started ☐ In progress ☐ Implemented ☐ Effective ☐ Not effective	
						☐ Open ☐ Controlled ☐ Accepted ☐ Closed	☐ Not started ☐ In progress ☐ Implemented ☐ Effective ☐ Not effective	
						☐ Open ☐ Controlled ☐ Accepted ☐ Closed	☐ Not started ☐ In progress ☐ Implemented ☐ Effective ☐ Not effective	
						☐ Open ☐ Controlled ☐ Accepted ☐ Closed	☐ Not started ☐ In progress ☐ Implemented ☐ Effective ☐ Not effective	
						☐ Open ☐ Controlled ☐ Accepted ☐ Closed	☐ Not started ☐ In progress ☐ Implemented ☐ Effective ☐ Not effective	

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Risk review log

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Study / site	SRR ID / revision	Revision effective date	Review log sheet no. / total

G Risk review log

One line per risk reviewed. Mark whether it is a scheduled review, a trigger or an urgent entry from H. Never overwrite an earlier line.

Date / reviewer	Risk ID	Review type	Trigger and evidence reviewed	Control effectiveness	Current priority and status	Decision / linked record	Next review
		☐ Scheduled ☐ Trigger ☐ Urgent (H)		☐ Effective ☐ Partly effective ☐ Not effective ☐ Not assessable			
		☐ Scheduled ☐ Trigger ☐ Urgent (H)		☐ Effective ☐ Partly effective ☐ Not effective ☐ Not assessable			
		☐ Scheduled ☐ Trigger ☐ Urgent (H)		☐ Effective ☐ Partly effective ☐ Not effective ☐ Not assessable			
		☐ Scheduled ☐ Trigger ☐ Urgent (H)		☐ Effective ☐ Partly effective ☐ Not effective ☐ Not assessable			
		☐ Scheduled ☐ Trigger ☐ Urgent (H)		☐ Effective ☐ Partly effective ☐ Not effective ☐ Not assessable			
		☐ Scheduled ☐ Trigger ☐ Urgent (H)		☐ Effective ☐ Partly effective ☐ Not effective ☐ Not assessable			
		☐ Scheduled ☐ Trigger ☐ Urgent (H)		☐ Effective ☐ Partly effective ☐ Not effective ☐ Not assessable			

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Urgent risks and decisions

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H Emerging or urgent risks

Act on the safety, PI or sponsor route at once; enter the risk in the register afterwards and review it in G. Clinical details stay in the SART or the source record.

No. · date / time	Detected by (role)	Local risk identified	Immediate containment and receiving record	PI notified	Register entry
				☐ Yes ☐ Pending ☐ Not required	☐ New risk ☐ Update ☐ Not required Risk ID:
				☐ Yes ☐ Pending ☐ Not required	☐ New risk ☐ Update ☐ Not required Risk ID:
				☐ Yes ☐ Pending ☐ Not required	☐ New risk ☐ Update ☐ Not required Risk ID:

I Status and escalation decisions

Decided and signed here; F is then updated. The PI decides on High and Critical risks and may delegate Low and Moderate.

Lower the priority: only after a review in G shows the controls work; a planned or completed action is not enough. **Accept the residual risk:** no further action is proportionate; record why and keep the trigger. **Close the risk:** the condition no longer applies or the vulnerability was removed with evidence; a closed CAPA alone does not close a risk. **Escalate:** to the CAPA log when a control failed or the problem recurs; to the PI (POT) or a milestone review (MRL) when a decision or a phase gate is affected; to the sponsor when the effect may reach beyond the site or the CIP requires it; to the SART when a safety event may be involved.

Date	Risk ID	Decision	Rule met and evidence	Route / record link	Decided by (PI or delegate)
		☐ Lower priority ☐ Accept residual ☐ Close ☐ Escalate			
		☐ Lower priority ☐ Accept residual ☐ Close ☐ Escalate			
		☐ Lower priority ☐ Accept residual ☐ Close ☐ Escalate			
		☐ Lower priority ☐ Accept residual ☐ Close ☐ Escalate			

J Register review certification

At each review of the whole register. The site sets how often this happens.

Review date		**Review frequency set by the site**	
Open High / Critical risks			
Accepted residual risks			
Next scheduled SRR review		**Linked POT / MRL review ID**	

Overall assessment · what changed since the last review, where the site is most exposed, what needs attention next

I confirm that the risks in this register were reviewed against current evidence and the controls in place, and that actions, triggers and communications are assigned or linked. This does not mean the risks are gone.

Register owner

Name / role:

Signature / date:

Principal Investigator

Name:

Signature / date: